

The Station

Engaged with the evidence.

February 2026's Edition

TEAM HUDDLE

Welcome to The Station. This is our monthly digest of the most relevant new research, guideline updates, and key debates from departmental teaching. We know clinical work doesn't always allow you to make every session, so this brings the main takeaways into one place. The aim is simple: help the whole team stay current with the evidence that shapes our practice.

January Theme: Preterm Birth

During last month's talks, we discussed preterm birth, which affects around 8% of UK births and accounts for about 75% of early neonatal deaths, with significant long-term morbidity. This edition recaps the evidence discussed, from the limits of screening to the longer-term impacts of our interventions, to keep practice aligned with the best data.

WEEK 1 • CTG

No session held

Universal Screening for Preterm Birth in Low-Risk Women (IPP Trial)

Am J Obstet Gynecol MFM, 2024 • Presented by **Khalid Shamiyah**

KEY FINDINGS

This RCT (n=1,334) investigated universal transvaginal ultrasound screening at 18–24 weeks in asymptomatic, low-risk women. Women with a short cervix ($\leq 25\text{mm}$) received vaginal progesterone and a cervical pessary.

KEY RESULTS

Result: There was no significant reduction in preterm birth <37 weeks (7.5% screening vs 8.7% control). The prevalence of short cervix in the low-risk population was too low (1.9%) to make this an effective screening target.

TAKE HOME

Routine universal cervical length screening in low-risk women is not currently supported by evidence, aligning with current national guidance.

[Link to paper ↗](#)

Antenatal Corticosteroids (RCOG GTG No. 74)

Presented by **Martha Shana**

24+0 to 34+6 weeks:

Recommended where preterm birth is imminent (within 1–7 days).

35+0 to 36+6 weeks:

Routine use not advised. Emphasise balanced discussion weighing RDS reduction against neonatal risks (hypoglycaemia, long-term neurodevelopmental impact).

Planned CS (37+0 to 38+6 weeks):

Do not routinely offer. Discuss uncertainties around benefits and risks with parents.

[Link to guideline ↗](#)

Antenatal Corticosteroids & Mental/Behavioural Disorders

JAMA, 2020 • Presented by Kianoush Zarrindej

KEY FINDINGS

Finnish study (~670k children) found antenatal steroids linked to increased mental/behavioural disorders, even at term—suggesting unnecessary exposure carries neurodevelopmental risk without respiratory benefit.

KEY RESULTS

Balance immediate respiratory benefits against long-term neurodevelopmental risks. Direct parents to RCOG patient information leaflet. Defer elective CS to 39+ weeks when possible.

TAKE HOME

Counsel parents using evidence and RCOG guidance for informed decision-making.

[Link to paper ↗](#)

HISTORICAL PERSPECTIVE

From Chickens to Children

Baker JP. J Perinatol. 2000.

The modern neonatal incubator was actually inspired by a trip to the zoo. In 1880, French obstetrician Stéphane Tarnier visited the Paris Zoo and observed incubators being used to hatch chicks. Realising that temperature regulation was the missing link for premature babies, he had the zoo's instrument-maker build a similar device for the Paris Maternité hospital. This first "couveuse" reduced the mortality rate of infants between 1.2kg and 2kg from 66% to 38% almost overnight.



Tarnier's incubators in the Maternité Hospital, Paris, 1884. Source: Illustrated London News, 8 March 1884, p. 228.

[View on PubMed ↗](#)

Upcoming Meetings

DATE	TIME	EVENT	TOPIC	LOCATION
Feb 3 Tuesday	08:30–09:00	CTG	CTG meeting (Zoe Slack)	Microsoft Teams
Feb 10 Tuesday	08:30–09:00	Gynae JC	PTSD following a pregnancy loss (Shehani Alwis)	Microsoft Teams
Feb 17 Tuesday	08:30–09:00	Guidelines	NICE Ectopic pregnancy and miscarriage: diagnosis and initial management (Aashlesha Sardesai)	Microsoft Teams
Feb 24 Tuesday	08:30–09:00	Obs JC	TBD (Lucy Thorn)	Microsoft Teams

REMINDER

MDT Teaching

Tuesday morning 8:30–09:00

